

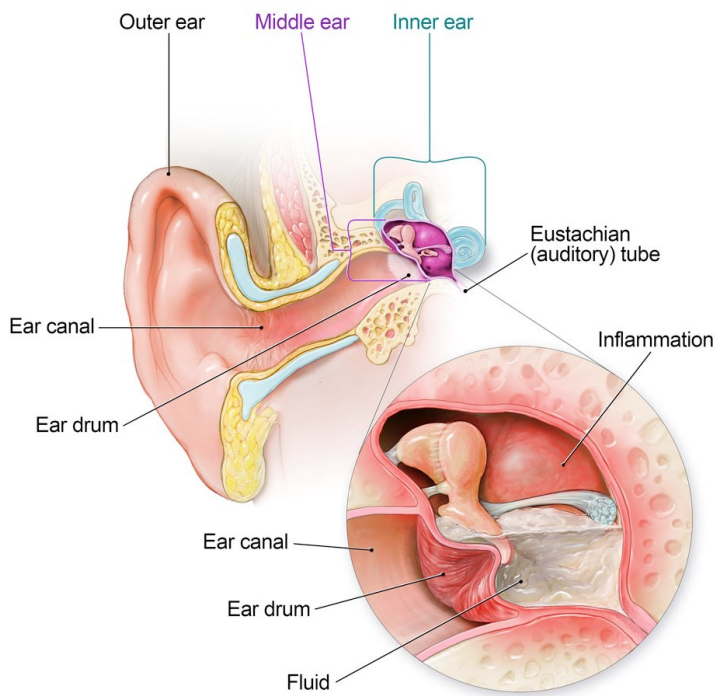


Treating Pediatric Ear Pain

A healthy 10-year-old boy presents via telephone appointment with left-side ear pain that began yesterday, 3/10 pain scale, and improves with ibuprofen. He's afebrile, but he has had a mild cough and nasal congestion for 2-3 days. Per the mother, his ears look normal externally, no pain to touch, and no recent swimming. He is otherwise playful, eating and drinking well, and has no shortness of breath. He's COVID antigen negative.

What is the *next* best step?

- Immediately recommend a same-day appointment to rule-out otitis media
- Prescribe amoxicillin for possible otitis media
- Prescribe otic antibiotic drops to appease the parent
- Via joint decision-making with the parent, offer a watchful waiting approach, supportive care measures, and give return precautions



Note. From *Healthy Ear/Infection of Middle Ear* [Infographic], by Centers for Disease Control and Prevention, n.d., CDC (<https://www.cdc.gov/antibiotic-use/ear-infection.html>).

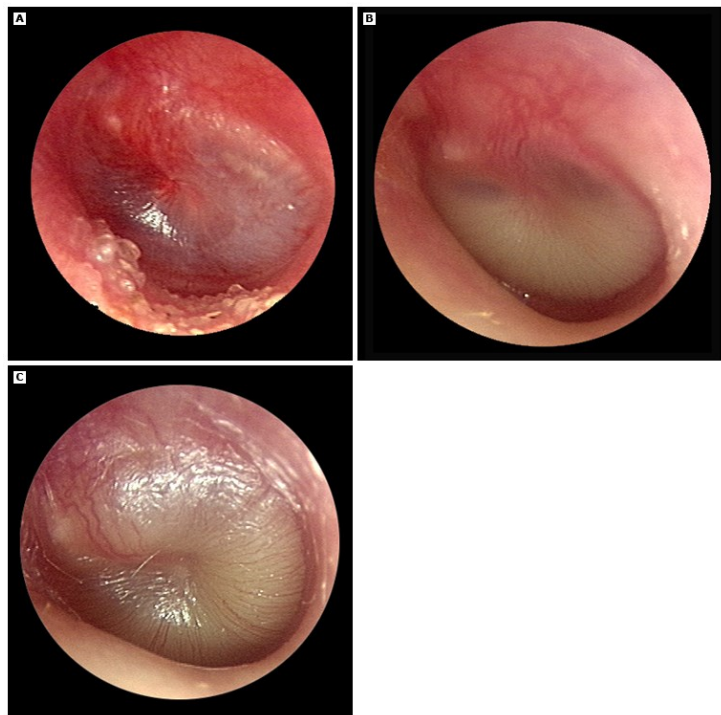
Best answer: d. Via joint decision-making with the parent, offer a watchful waiting approach, supportive care measures, and give return precautions.

Even for otitis media confirmed in person, [AAP Guidelines](#) recommend offering a watchful waiting approach (without antibiotics) for select children, based on joint decision-making with the parent.

For this reason, during virtual encounters, we should also offer watchful waiting for select groups.

Virtual Interpretation of AAP Guidelines for Otitis Media

- We recommend offering a *watchful waiting* approach for children > 6 months of age with mild otalgia and mild illness (e.g., mild otalgia < 48 hours, temp < 102.2°F (39°C), fever < 48-72 hours).
 - Watchful waiting: Supportive care with ibuprofen or acetaminophen for 48 hours. In-person follow-ups as needed if symptoms persist or worsen.
- When otitis media is suspected, we recommend *in-person* evaluation for the following groups:
 - Children < 6 months
 - Any child that is ill-appearing
 - Moderate to severe otalgia or persistent otalgia > 48 hours
 - Fever ≥ 102.2°F (39°C) or fever > 48-72 hours
 - Children < 24 months with BILATERAL otalgia
 - If parent declines the watchful waiting approach



Note. From *Appearance of the tympanic membrane in three children with acute otitis media* [Photographs], by Glenn C. Isaacson, 2023, UpToDate (<https://www.uptodate.com/contents/image?csi=fd6adce2-0c57-4b93-a994-beef1a214667&source=contentShare&imageKey=PEDS%2F97608>).

***If patient has a digital otoscope and can upload clear images of tympanic membrane(s), then treating otitis media virtually would be a viable option.**

Why Shouldn't We Choose the Other Options?

- Immediately recommending a same-day appointment is not unreasonable. However, given this patient is an older child with mild symptoms, we should first discuss watchful waiting.
- Empirically prescribing oral antibiotics for otitis media in children via telephone/video appointment without a physical examination goes against standard of care.
- Virtually prescribing otic antibiotic drops for otitis externa is reasonable when history is highly suggestive of swimmer's ear. However, this patient's history is not suggestive at all.

References

- Centers for Disease Control and Prevention. (n.d.). *Healthy Ear/Infection of the Middle Ear* [Infographic]. Centers for Disease Control and Prevention. <https://www.cdc.gov/antibiotic-use/ear-infection.html>
- Goguen, L. A. (2019). External otitis: Treatment. UpToDate. <https://www.uptodate.com/contents/external-otitis-treatment>
- Greenes, D. (2020). Evaluation of earache in children. UpToDate. <https://www.uptodate.com/contents/evaluation-of-earache-in-children>
- Isaacson, G. C. (2023). Appearance of the tympanic membrane in three children with acute otitis media [Photographs]. UpToDate. <https://www.uptodate.com/contents/image?csi=fd6adce2-0c57-4b93-a994-beef1a214667&source=contentShare&imageKey=PEDS%2F97608>
- Lieberthal, A. S., Carroll, A. E., Chonmaitree, T., Ganiats, T. G., Hoberman, A., Jackson, M. A., Joffe, M. D., Miller, D. T., Rosenfeld, R. M., Sevilla, X. D., Schwartz, R. H., Thomas, P. A., & Tunkel, D. E. (2013). The diagnosis and management of acute otitis media. *Pediatrics*, 131(3), 964-999. <https://doi.org/10.1542/peds.2012-3488>
- Lieu, V. (2001). VMC pediatric virtual playbook: Ear pain. VMC SCAL Peds. <https://sp-cloud.kp.org/sites/VMCSVALPeds/SitePages/Ear-Pain---Pediatric-Virtual-Playbook.aspx>
- Ray, K. N., Shi, Z., Gidengil, C. A., Poon, S. J., Uscher-Pines, L., & Mehrotra, A. (2019). Antibiotic prescribing during pediatric direct-to-consumer telemedicine visits. *Pediatrics*, 143(5). <https://doi.org/10.1542/peds.2018-2491>